

Surgical Pathology Report

Name: [REDACTED]
DOB: [REDACTED]
Gender: F
MRN: [REDACTED]
Location:
Physician:

Case #: [REDACTED]
Collected:
Received:
Reported:
Copy To:

Pathologic Interpretation:

- A. Sentinel node #1 Count # FS:
- Metastatic carcinoma to lymph node (1/1) with extracapsular extension.
- B. Sentinel node #2 count IS:
- No malignancy seen in one lymph node (0/1).
- Immunohistochemistry for keratin to follow.
- C. Sentinel node #3 count
- No malignancy seen in one lymph node (0/1).
- Immunohistochemistry for keratin to follow.
- D. Sentinel node #4 count FS:
- No malignancy seen in one lymph node (0/1).
- Immunohistochemistry for keratin to follow.
- E. Sentinel node #5 count FS:
- No malignancy seen in one lymph node (0/1).
- Immunohistochemistry for keratin to follow.
- F. Sentinel node #6 count FS:
- No malignancy seen in one lymph node (0/1).
- Immunohistochemistry for keratin to follow.
- G. Axillary fat (perm):
- No malignancy seen in one lymph node (0/1).
- H. Skin right breast:
- Skin with no specific pathologic change.
- I. Right breast lumpectomy (short stitch superior, long lateral) fresh:
- Invasive poorly differentiated ductal carcinoma, Nottingham grade 3 (3 + 3 + 3), 3.5 cm.
- Lymphovascular space invasion is not identified.
- Specimen margins are negative for tumor.
- J. Additional deep margin (perm):
- No malignancy seen.
- K. Right axillary contents (perm):
- No malignancy seen in ten lymph nodes (0/10).
- L. Left breast tissue (perm):
No malignancy seen.

100-0-3

Carcinoma, infiltrating ductal, NOS
Site: breast, NOS 050.9 2/8/11

8500/3

UUID: E8B860F6-F3FD-4FDF-8467-8988646C2F00
TCGA-EW-A1J1-01A-PR

Redacted



Surgical Pathology Tumor Summary

Specimen: partial breast

Procedure: Excision without wire-guided localization

Lymph node sampling: Sentinel lymph nodes, axillary dissection (partial or complete dissection)

Specimen Integrity: Single intact specimen

Specimen Size: Greatest dimension: 9.5 cm

Additional dimensions: 8 x 5.5 cm

Specimen Laterality: Right

Tumor Size: Greatest dimension of largest focus of invasion over 0.1 cm: 3.5 cm

Additional dimensions: 3 x 3 cm

Skin: Invasive carcinoma does not invade into the dermis or epidermis

Skeletal Muscle: No skeletal muscle present

SURGICAL PATHOL Report

Histologic type of invasion: Invasive ductal carcinoma.

Glandular (Acinary/tubular Differentiation: Score 3: <10% of tumor are forming glandular/tubular structures

Nuclear Pleomorphism: Score 3: Vesicular nuclei, often with prominent nucleoli exhibiting marked variation in size and shape, occasionally with very large and bizarre forms.

Mitotic Count: Score 3

Overall Grade: Grade 3: scores of 8 or 9

Margins uninvolved by invasive carcinoma

Lymph-vascular invasion: Not identified

Number of lymph nodes examined: 8

Total number of lymph nodes examined (sentinel and non-sentinel): 17

Number of lymph nodes with macrometastasis (>0.2 cm): 1

Extranodal extension: Present

Method of evaluation of sentinel lymph nodes: Hematoxylin and eosin (H & E) one level, immunohistochemistry

PT2: Tumor >20 mm but <50 mm in greatest dimension

PN1a: Metastases in 1 to 3 axillary lymph nodes, at least 1 metastasis greater than 2.0 mm

Distant metastasis: not applicable

Estrogen Receptor: Performed on another specimen

, immunoreactive tumor cells present (>1%)

Progesterone Receptor: Performed on another specimen

, immunoreactive tumor cells present (>1%)

Immunoperoxidase studies: Performed on another specimen

, negative (score 0)

Electronically Signed Out By

NOTE: Some immunohistochemical antibodies are analyte specific reagents (ASRs) validated by our laboratory (Her 2, Papan, H. pylori, HBcore). These ASRs are clinically useful indicators that do not require FDA approval. These clones are used: ID5=ER, PgR 636=PR, A485=HER2, H-11=EGFR, CCH2/DDG8=CMV, F39.4.1=AR and HPV by ISH. All immunohistochemical stains are used with formalin or molecular fixed, paraffin embedded tissue. Detection is by LSAB. The results are read by a pathologist as positive or negative.

As the attending pathologist, I attest that I: (i) Examined the relevant preparation(s) for the specimen(s); and (ii) Rendered the diagnosis(es).

Procedures/Addenda

Addendum

Date Ordered:

Status: Signed Out

Date Complete:

Date Reported:

Addendum Diagnosis

- B. Immunohistochemistry for Keratin stain is negative for metastatic carcinoma to the lymph node.
- C. Immunohistochemistry for Keratin stain is negative for metastatic carcinoma to the lymph node.
- D. Immunohistochemistry for Keratin stain is negative for metastatic carcinoma to the lymph node.
- E. Immunohistochemistry for Keratin stain is negative for metastatic carcinoma to the lymph node.

Intraoperative Consultation

AFS. Sentinel node #1 Count # FS:
One lymph node with metastatic carcinoma.

BFS. Sentinel node #2 count FS:

SURGICAL PATHOL Report

One lymph node, no carcinoma seen.

CFS. Sentinel node #3 count
Adipose tissue only. No lymph node identified grossly.

DFS. Sentinel node #4 count : FS:
One lymph node, no carcinoma seen.

EFS. Sentinel node #5 count / FS:
One lymph node, no carcinoma seen.

FFS. Sentinel node #6 count FS:
One lymph node, no carcinoma seen.

Clinical History:

Patient is a female with right breast cancer.

Pre Operative Diagnosis:

Right breast cancer.

Specimen(s) Received:

- A: Sentinel node #1 Count # FS
- B: Sentinel node #2 count FS
- C: Sentinel node #3 count FS
- D: Sentinel node #4 count : FS
- E: Sentinel node #5 count FS
- F: Sentinel node #6 count FS
- G: Axillary fat (perm)
- H: Skin right breast
- I: Right breast lumpectomy (short stitch superior, long lateral) fresh
- J: Additional deep margin (perm)
- K: Right axillary contents (perm)
- L: Left breast tissue (perm)

Gross Description:

- A. Received fresh and labeled "sentinel node #1 count # 0.2 cm. The cassettes are submitted as follows: FS" consists of pale yellow soft tissue measuring 1 x 0.3 x 0.2 cm. The cassettes are submitted as follows:
 - 1 Submitted for frozen section
 - 2 Remainder of the tissue
- B. Received fresh and labeled "sentinel node #2 count x 0.2 x 0.3 cm in aggregate. The cassettes are submitted as follows: FS" consists of light brown fragments —49—count measuring 1 x 0.2 x 0.3 cm in aggregate. The cassettes are submitted as follows:
 - 1 Submitted for frozen section
 - 2 Remainder of the tissue.
- C. Received fresh and labeled "sentinel node #3 count specimen submitted in toto in two cassettes. ' consists of light brown soft tissue measuring 2 x 1 x 1 cm. The
- D. Received fresh and labeled "seninel node #4 count Specimen as follows: FS" consists of pale white tissue measuring 1 x 0.3 x 0.2 cm.
 - 1 Submitted for frozen section.
 - 2 Remainder of tissue
- E. Received fresh and labeled "sentinel node #5 count! The specimen is submitted as follows: FS" consists of sift white fragment measuring 1 x 0.3 x 0.2 cm.
 - 1 Submitted for frozen section

SURGICAL PATHOL Report

2 Remainder of tissue.

- F. Received fresh and labeled "sentinel node #6 count FS" consists of light brown fragment measuring 0.5 x 0.3 x 0.2 cm. The specimen is submitted as follows:
- 1 Submitted for frozen section
 - 2 Remainder of tissue
- G. Received in formalin and labeled "axillary fat (perm)" consists of yellow brown soft tissue measuring 4 x 2 x 3 cm in aggregate. The entire specimen is submitted in toto in eight cassettes.
- H. Received in formalin and labeled "skin right breast" consists of skin tag measuring 5 x 0.5 x 0.1 cm. A representative section of the skin tag is submitted in one cassette.
- I. Received in formalin and labeled "right breast lumpectomy (short stitch superior, long lateral) fresh" consists of breast tissue mass measuring 186 grams and measures 9.5 cm medial to lateral, 5.5 cm inferior superior and 8 cm anterior to posterior. It is round to oval in shape. It is yellow in color and soft in consistency. The specimen has sutures for orientation. The specimen is inked as follows: anterior yellow; superior blue; posterior black; inferior green; lateral orange and medial red. Serial sectioning of the specimen reveals a well defined round to oval mass measuring 3 x 3 x 3.5 cm. It is hard in consistency. It occupies 20% of the entire specimen. The nearest margin is at the superior margin which is 1.5 cm from the mass. Remaining margins are away from the mass. There is an area of fibrosis which is situated anterior to the mass; it is 1 cm away from the mass. The area of fibrosis measures 1 x 1 cm. The breast parenchyma in between the mass and the fibrosis is normal. The remaining breast parenchyma consists of the fat to stroma ratio 90:10. The cassettes are submitted as follows:
- 1 Superior margin
 - 2 Inferior margin
 - 3 Anterior margin
 - 4 Posterior margin
 - 5 Medial margin
 - 6 Lateral margin
 - 7-10 Representative sections of the tumor
 - 11 Area of fibrosis
 - 12 Breast parenchyma between the tumor and fibrosis.
- J. Received in formalin and labeled "additional deep margin (perm)" consists of two yellow fragments soft in consistency measuring 1 x 0.3 x 0.3 cm in aggregate. The entire specimen is submitted in toto in two cassettes.
- K. Received in formalin and labeled "right axillary contents (perm)" consists of a yellow brown soft tissue measuring 9 x 7 x 2 cm. Ten possible lymph nodes are extracted from the specimen and submitted in six cassettes. One lymph node which measures 1 x 0.5 x 0.3 cm, bisected and submitted in cassette 1.
- 2 One lymph node bisected.
 - 3 Two lymph nodes
 - 4&5 Three lymph nodes
 - 6 One lymph node
- L. Received in formalin and labeled "left breast tissue (perm)" consists of a breast mass measuring 465 grams and measures 8 x 5 x 2 cm in greatest dimension. It has a skin ellipse which measures 4 x 1.5 cm. The entire specimen other than skin is inked black. Serial sectioning of the mass it reveals fibrofatty tissue with fat to stoma ratio 70:30. No gross lesions are identified.
- 1-5 Representative section of the fibrofatty tissue
 - 6 Skin tag

ICD-9(s): 174.8 196.3

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 12/11	